# INDIAN PHARMACOPOEIA COMMISSION (IPC)

*Pharmacovigilance Programme of India (PvPI) — Guidance for Marketing Authorisation Holders*
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## SECTION 4: TRIAGE, SERIOUSNESS AND EXPECTEDNESS CRITERIA

**[Clause IPC-PVPI-G4.1] Seriousness Evaluation Criteria**
Under national pharmacovigilance standards, an adverse drug event is deemed SERIOUS if it results in any of the following outcomes: (1) Patient Death; (2) Life-threatening condition placing the patient at immediate risk of demise; (3) Inpatient Hospitalization or prolongation of existing hospitalization; (4) Persistent or significant disability or incapacity; (5) Congenital anomaly or birth defect; or (6) Other Medically Important Event requiring medical or surgical intervention to prevent one of the outcomes listed above.

**[Clause IPC-PVPI-G4.2] Expectedness and Listedness Determination**
An adverse event is categorized as EXPECTED (Listed) when its specificity, nature, severity, and clinical outcome are consistent with the approved reference safety information (Product Information / Package Insert / SmPC) authorized by the Indian Regulatory Authority. Conversely, an event is UNEXPECTED if it is not specifically named in the package insert, or if its observed severity, complication, or clinical presentation exceeds the documented safety profile.

**[Clause IPC-PVPI-G4.3] Layperson Language Mapping and Clinical Synonyms**
Spontaneous reports from patients and consumers frequently employ everyday vernacular rather than formal MedDRA terminology. Intake specialists and algorithms must cross-reference colloquial terminology with verified medical entries: - 'skin peeling badly', 'skin shedding', or 'skin falling off' -> Exfoliative dermatitis, Epidermal necrolysis, Stevens-Johnson syndrome (SJS), or Toxic epidermal necrolysis (TEN). - 'turning yellow' -> Jaundice, hepatic impairment, hyperbilirubinemia. - 'cannot breathe' -> Dyspnea, bronchospasm, anaphylactic reaction.

## SECTION 5: REPORTING DECISION LOGIC AND TIMELINES

**[Clause IPC-PVPI-G5.1] Triage Routing Rules**
Routing determination must synthesize seriousness, expectedness, and qualifying modifiers: (a) Serious and Unexpected: Expedited Reporting within fourteen (14) calendar days to NCC-PvPI and CDSCO. (b) Serious and Expected: Periodic Safety Update Reporting (PSUR) unless triggering Section 5.2.4. (c) Non-Serious: Periodic Safety Update Reporting or internal safety archive.

**[Clause IPC-PVPI-G5.2.4] Special Safety Modifier for Severe Cutaneous Adverse Reactions (SCAR)**
CRITICAL SAFETY OVERRIDE: Any adverse drug event manifesting as a Severe Cutaneous Adverse Reaction (SCAR) — including Stevens-Johnson Syndrome (SJS), Toxic Epidermal Necrolysis (TEN), or severe exfoliative dermatitis — accompanied by inpatient hospital admission, constitutes an emergent drug safety signal. Such reports MUST BE ROUTED AS EXPEDITED REPORTS within fourteen (14) calendar days, even when the reaction is catalogued as an adverse effect in the approved package insert.

**[Clause IPC-PVPI-G7.1] Human Pharmacovigilance Escalation Mandate**
Automated systems must escalate the case for Human Pharmacovigilance Review under the following circumstances: (1) The suspect drug is outside the authorized indexed reference corpus; (2) The reported reaction description cannot be matched with confidence to product information; (3) The chronology or latency between drug initiation and adverse event onset is ambiguous or biologically implausible; (4) The governing regulatory provisions or citations contain contradictory requirements. An automated determination must never be returned without verified citations from the reference corpus.